National Alliance for Care at Home

June 13, 2025

The Honorable Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
7500 Security Boulevard
Baltimore, MD 21244–1850

**RE: CMS-0042-NC, Request for Information; Health Technology Ecosystem**

Dear Administrator Oz:

The National Alliance for Care at Home (the Alliance) appreciates the opportunity to submit comments on the Centers for Medicare & Medicaid Services (CMS**) Request for Information (RFI): Health Technology Ecosystem (CMS–0042–NC).** The Alliance is the unified voice for providers delivering high-quality, person-centered healthcare to individuals, wherever they call home. Our members are providers of different sizes and types—from small rural agencies to large national companies—including government-based providers, nonprofit organizations, systems-based entities, and public corporations. Our members, including over 1,500 providers representing 10,000 offices and locations, serve over 4 million patients nationwide through a dedicated workforce of over 1 million employees, staff, and volunteers. Formed through the joint affiliation of the National Association for Home Care & Hospice (NAHC) and the National Hospice and Palliative Care Organization (NHPCO), the Alliance is dedicated to advancing policies that support care in the home for millions of Americans at all stages of life, individuals with disabilities, persons with chronic and serious illnesses, as well as dying Americans who depend on those supports.

We appreciate CMS's efforts to modernize our nation's digital health system to enhance innovative care access and develop a robust patient-centered digital health infrastructure. Indeed, we share this common goal, but note there are a number of significant barriers in the home-based care space that blunt efforts to support certified electronic health record (EHR) adoption and interoperability in the post-acute space.

Our detailed comments are provided below.

# I. PR–7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem?

## A. Investment is Necessary to Facilitate Certified-EHR Adoption

The Alliance appreciates CMS's recognition of the critical importance of certified electronic health record (EHR) adoption and interoperability across the healthcare continuum. Effective, real-time data exchange is critical to achieving coordinated, high-quality patient care. Yet, home-based providers face significant challenges in adopting these capabilities due to limited financial and technical resources, a situation distinct from hospitals and large health systems that benefited from previous federal investments. In other words, lack of technology is not the problem—lack of resource availability is.

Adoption and interoperability among Medicare and Medicaid-participating providers, particularly home-based care, such as home health, hospice, and Medicaid home and community-based services (HCBS), lag far behind the acute and ambulatory sectors. The reason is simple: unlike hospitals and physicians, post-acute providers have never benefited from the concerted federal investment and policy drivers that make health information technology (HIT) adoption feasible.

In fact, a key reason or the slower adoption of interoperable HIT in long-term and post-acute care (LTPAC) settings is the exclusion of these providers from significant federal investments made under the 2009 Health Information Technology for Economic and Clinical Health (HITECH) Act.[1] This law established incentive payments specifically for eligible professional and hospitals participating in Medicare and Medicaid to adopt Office of the National Coordinator (ONC) for Health Information Technology-certified Health Information Technologies (CEHRT). Specifically, LTPAC providers, including home health agencies (HHAs) and hospice providers, were explicitly excluded from eligibility for EHR adoption incentives under Section 4101 of the HITECH Act.[2]

In other words, home health and hospice providers were not eligible for the substantial funding incentives provided through the HITECH Act, which significantly accelerated EHR adoption in acute and ambulatory settings through the establishment of the "Meaningful

[1] The HITECH Act of 2009, Pub. L. 111-5, was enacted on February 17, 2009 as part of the American Recovery and Reinvestment Act.
[2] *See* 42 U.S.C. §§ 1395w-4(o), 1395ww(n).

Use" program.[3] Consequently, many providers in the home-based care sector still struggle with the costs associated with obtaining, implementing, and maintaining certified EHR technology. This funding gap created an uneven playing field, leaving many post-acute providers without the resources to invest in certified EHRs and data exchange infrastructure.

Illustratively, a 2023 U.S. Department of Health and Human Services (HHS) report found that while approximately 80% of home health agencies and hospices had adopted at least basic EHRs by 2018, routine interoperability is "not widely used" without policy drivers or levers. As HHS stated, "[a]lthough there is broad consensus that LTPACs and their patients would benefit from greater access to HIT and information exchange, results from our interviews with SMEs suggest that those expectations do not match LTPAC financial resources. . . . **[U]nlike with the Meaningful Use/Promoting Interoperability program, there are no levers or drivers requiring LTPAC providers to share information with hospitals and office-based settings**."[4] (emphasis added).

**Recognizing Congressional action may be necessary for substantial new funding, we urge CMS to leverage its existing authority to provide upfront financial and technical assistance.** These resources are critical to support the purchase, implementation, and ongoing maintenance of interoperable HIT, particularly if CMS intends to mandate CEHRT adoption in the future. Importantly, any new interoperability mandates must be paired with funding and support. Most home-based providers currently use EHR systems purchased entirely through their own resources, without the benefits of federal funding or incentives. Consequently, many of these systems do not meet ONC-certification criteria and would require significant and costly upgrades if future CEHRT mandates are implemented. Providers will also face additional expenses related to staffing and training necessary to manage these advanced technology systems. Therefore, dedicated incentives designed to offset these incremental costs would strongly support sustained adoption and interoperability efforts.

Encouraging alignment and coordination with hospital systems and other referral partners through incentives would also greatly enhance interoperability and facilitate smoother care transitions, leading to improved patient care outcomes, satisfaction, and better informed care delivery.

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[3] "To qualify for incentive payments through [CMS] EHR Incentive Programs, eligible providers and hospitals must demonstrate meaningful use of an electronic health record (EHR). In other words, "meaningful use" sets the specific objectives that eligible professionals and hospitals must achieve to participate in the EHR Incentive Programs." https://www.healthit.gov/faq/what-meaningful-use.

[4] https://aspe.hhs.gov/sites/default/files/documents/c39b80481327bb3136a07f2c0afd0f65/hit-adoption-utilization-ltpac-settings.pdf.

## B. Rural and Small Home-Based Care Providers Experience Unique Challenges

As CMS advances interoperability in post-acute care, health access considerations must remain front and center. Just as there are disparities in patient access to care, there are disparities in provider access to health IT infrastructure. Rural and small home-based care providers face even greater barriers to adopting ONC-certified CEHRT, given their limited resources and smaller operational scale. Specific challenges these providers face include limited broadband access, unreliable internet connectivity, geographical isolation with long travel times, and difficulty recruiting and retaining technical personnel. These providers often lack capital for major HIT investments; they may struggle to recruit staff or afford full-time technology personnel; and many in rural areas still contend with limited broadband connectivity. If policy solutions do not account for these differences, we risk widening the digital divide.

In supporting small and rural agencies, we note that these providers are often the only sources of care in their communities. They deserve the same modern tools as anyone else, as do the patients these providers serve. Federal investments can ensure that rural home health and hospice clinicians have access to their patients' hospital and provider records at the touch of a button, thereby informing appropriate and timely care delivery.

We encourage CMS to identify opportunities to provide targeted financial support and grants aimed at rural and smaller home-based care providers and explore opportunities to coordinate closely with complementary federal initiatives, such as the U.S. Department of Agriculture[5] and Federal Communications Commission[6] broadband projects, to strengthen infrastructure in support of a reliable data exchange.

## C. Data Interoperability Challenges and Opportunities

Understanding interoperability's tangible impact on patient care is critical. Federal initiatives like the "Meaningful Use" program successfully drove widespread adoption of EHR systems in acute and ambulatory care settings through structured incentives linked to clear performance milestones.

[5] https://www.rd.usda.gov/programs-services/telecommunications-programs
[6] https://www.fcc.gov/general/national-broadband-plan

CMS should leverage lessons learned from this program by identifying opportunities to support investments necessary to enable broad adoption and successful integration of these technologies into practice in the home-based care space. For example, financial incentives could support the adoption of CEHRT and participation in health information exchanges. CMS could subsequently evaluate adoption patterns and patient care impacts to guide future efforts.

It is important to note that any measures must accommodate the operational realities of home health, hospice, and Medicaid HCBS services, in consideration of the unique care needs for these patient populations in the home-based care environment. To this end, CMS could explore demonstration projects and programs to systematically evaluate how enhanced data sharing and CEHRT adoption affect patient outcomes, coordination of care, and overall patient satisfaction in home-based care environments.
We are optimistic that with the right mix of incentives, requirements, and support, the coming years will see a transformation in how home-based care providers use technology to connect and care for some of our nation's most vulnerable patient populations. CMS has an opportunity to truly unify the healthcare ecosystem by bringing care-at-home providers into the fold. HIT exists and continues to improve—what's needed is policy to drive adoption and connectivity at scale.

## II. Conclusion

The Alliance values CMS's commitment to improving technology to enhance beneficiary care. We reiterate our strong support for CMS's focus on the health technology ecosystem and emphasize the importance of including post-acute care, and particularly, home-based care, in this dialogue. We look forward to continued engagement on this important initiative to facilitate continued innovation in the care at home space. If you have questions or would like to schedule a meeting, your staff should feel free to contact me at slevy@allianceforcareathome.org.

Sincerely,

Scott Levy
Chief Government Affairs Officer
National Alliance for Care at Home